Drug Coverage Policy

Effective Date……05/01/2026
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Policy Title…… Adalimumab Products Prior Authorization Policy

Inflammatory Conditions – Adalimumab Products Prior Authorization Policy

- Abrilada™ (adalimumab-afzb subcutaneous injection – Pfizer)
- adalimumab-aacf subcutaneous injection (Fresenius Kabi)
- adalimumab-aaty subcutaneous injection (Celltrion)
- adalimumab-adaz subcutaneous injection (Sandoz/Novartis)
- adalimumab-adbm subcutaneous injection (Boehringer Ingelheim)
- adalimumab-bwwd subcutaneous injection (Cordavis)
- adalimumab-fkjp subcutaneous injection (Mylan)
- adalimumab-ryvk subcutaneous injection (Teva/Alvotech)
- Amjevita® (adalimumab-atto subcutaneous injection – Amgen)
- Cyltezo® (adalimumab-adbm subcutaneous injection – Boehringer Ingelheim)
- Hadlima™ (adalimumab-bwwd subcutaneous injection – Organon/Samsung Bioepis)
- Hulio® (adalimumab-fkjp subcutaneous injection – Mylan)
- Humira® (adalimumab subcutaneous injection – AbbVie, Cordavis)
- Hyrimoz® (adalimumab-adaz subcutaneous injection – Sandoz/Novartis, Cordavis)
- Idacio® (adalimumab-aacf subcutaneous injection – Fresenius Kabi)
- Simlandi® (adalimumab-ryvk subcutaneous injection – Teva/Alvotech)
- Yuflyma® (adalimumab-aaty subcutaneous injection – Celltrion)
- Yusimry™ (adalimuamb-aqvh subcutaneous injection – Coherus)

INSTRUCTIONS FOR USE
The following Coverage Policy applies to health benefit plans administered by Cigna Companies. Certain Cigna Companies and/or lines of business only provide utilization review services to clients and do not make coverage determinations. References to standard benefit plan language and coverage determinations do not apply to those clients. Coverage Policies are intended to provide guidance in interpreting certain standard benefit plans administered by Cigna Companies. Please note, the terms of a customer's particular benefit plan document [Group Service Agreement, Evidence of Coverage, Certificate of Coverage, Summary Plan Description (SPD) or similar plan document] may differ significantly from the standard benefit plans upon which these Coverage Policies are based. For example, a customer's benefit plan document may contain a specific exclusion related to a topic addressed in a Coverage Policy. In the event of a conflict, a customer's

benefit plan document always supersedes the information in the Coverage Policies. In the absence of a controlling federal or state coverage mandate, benefits are ultimately determined by the terms of the applicable benefit plan document. Coverage determinations in each specific instance require consideration of 1) the terms of the applicable benefit plan document in effect on the date of service; 2) any applicable laws/regulations; 3) any relevant collateral source materials including Coverage Policies and; 4) the specific facts of the particular situation. Each coverage request should be reviewed on its own merits. Medical directors are expected to exercise clinical judgment where appropriate and have discretion in making individual coverage determinations. Where coverage for care or services does not depend on specific circumstances, reimbursement will only be provided if a requested service(s) is submitted in accordance with the relevant criteria outlined in the applicable Coverage Policy, including covered diagnosis and/or procedure code(s). Reimbursement is not allowed for services when billed for conditions or diagnoses that are not covered under this Coverage Policy (see "Coding Information" below). When billing, providers must use the most appropriate codes as of the effective date of the submission. Claims submitted for services that are not accompanied by covered code(s) under the applicable Coverage Policy will be denied as not covered. Coverage Policies relate exclusively to the administration of health benefit plans. Coverage Policies are not recommendations for treatment and should never be used as treatment guidelines. In certain markets, delegated vendor guidelines may be used to support medical necessity and other coverage determinations.

OVERVIEW

Adalimumab products are tumor necrosis factor inhibitors (TNFis) approved for the following uses:[1]

- **Ankylosing spondylitis**, for reducing signs and symptoms in adults with active disease.
- **Crohn's disease**, for treatment of moderately to severely active disease in patients ≥ 6 years of age.
- **Hidradenitis suppurativa**, for the treatment of moderate to severe disease in patients ≥ 12 years of age.
- **Juvenile idiopathic arthritis (JIA)**, alone or in combination with methotrexate, for reducing signs and symptoms of moderately to severely active polyarticular disease in patients ≥ 2 years of age.
- **Plaque psoriasis**, for the treatment of adults with moderate to severe chronic disease who are candidates for systemic therapy or phototherapy and when other systemic therapies are medically less appropriate.
- **Psoriatic arthritis (PsA)**, alone or in combination with non-biologic disease-modifying antirheumatic drugs (DMARDs), for reducing the signs and symptoms of active arthritis, inhibiting the progression of structural damage, and improving physical function in adults with active disease.
- **Rheumatoid arthritis**, alone or in combination with methotrexate or other non-biologic DMARDS, to reduce the signs and symptoms, induce major clinical response, inhibit the progression of structural damage, and improve physical function in adults with moderately to severely active disease.
- **Ulcerative colitis**, for treatment of moderately to severely active disease in patients ≥ 5 years of age. However, efficacy has not been established in patients with ulcerative colitis who have lost response or were intolerant to another TNFi.
- **Uveitis**, in patients ≥ 2 years of age with noninfectious intermediate, posterior, and panuveitis.

Guidelines

TNFis are featured prominently in guidelines for treatment of inflammatory conditions.

- **Ankylosing Spondylitis and Spondyloarthritis:** Guidelines for ankylosing spondylitis and non-radiographic axial spondylitis are published by the American College of

Rheumatology (ACR)/Spondylitis Association of America/Spondyloarthritis Research and Treatment Network (2019).[3] TNFis are recommended as the initial biologic. In those who are secondary non-responders to a TNFi, a second TNFi is recommended over switching out of the class.

- **Crohn's Disease:** The American College of Gastroenterology (ACG) [2025] and the American Gastroenterological Association (AGA) [2025] have guidelines for the management of CD in adults.[4,17] Both guidelines recommend upfront use of advanced therapies, rather than step-up therapy after failure of corticosteroids and/or immunomodulators. Advanced therapies recommended include TNF inhibitors, Entyvio® (vedolizumab IV infusion, SC injection), interleukin (IL)-23 inhibitors, IL-12/23 inhibitors, and Rinvoq® (upadacitinib extended-release tablets).
- **Hidradenitis Suppurativa:** North American guidelines (2019) recommend consideration of adalimumab (level A strength of recommendation, level 1 evidence) for those with moderate to severe disease who do not respond to conventional therapy.[19]
- **JIA:** There are guidelines from ACR and the Arthritis Foundation for the treatment of JIA (2021) which address oligoarthritis and temporomandibular joint (TMJ) arthritis. For oligoarthritis, a biologic is recommended following a trial of a conventional synthetic DMARD.[6] In patients with TMJ arthritis, scheduled nonsteroidal anti-inflammatory drugs (NSAIDs) and/or intra-articular glucocorticoids are recommended first-line. A biologic is a therapeutic option if there is an inadequate response or intolerance. Additionally, rapid escalation to a biologic ± conventional synthetic DMARD (methotrexate preferred) is often appropriate given the impact and destructive nature of TMJ arthritis. In these guidelines, there is not a preferred biologic that should be initiated for JIA. ACR guidelines (2019) are also available specifically for juvenile non-systemic polyarthritis, sacroiliitis, and enthesitis.[5] TNFis are the biologics recommended for polyarthritis, sacroiliitis, and enthesitis. Biologics are recommended following other therapies (e.g., following DMARDs for active polyarthritis or following an NSAID for active JIA with sacroiliitis or enthesitis). However, there are situations where initial therapy with a biologic may be preferred over other conventional therapies (e.g., if there is involvement of high-risk joints such as the cervical spine, wrist, or hip; high disease activity; and/or those judged to be at high risk of disabling joint damage).
- **Plaque Psoriasis:** Guidelines from the American Academy of Dermatologists and National Psoriasis Foundation (2019) recommend adalimumab as a monotherapy treatment option for adults with moderate to severe disease.[7]
- **PsA:** Guidelines from ACR (2018) recommend TNFis over other biologics for use in treatment-naïve patients with PsA and in those who were previously treated with an oral therapy.[8]
- **Rheumatoid Arthritis:** Guidelines from ACR (2021) recommend addition of a biologic or a targeted synthetic DMARD for a patient taking the maximum tolerated dose of methotrexate who is not at target.[2]
- **Ulcerative Colitis:** The AGA (2024) and ACG (2025) have clinical practice guidelines on the management of moderate to severe ulcerative colitis in adults.[9,10] In moderate to severe disease, systemic corticosteroids or advanced therapies may be utilized for induction of remission. Advanced therapies recommended include TNFis, Entyvio, IL-23 inhibitors, IL-12/23 inhibitors, sphingosine-1-phosphate (S1P) receptor modulators, and Janus kinase (JAK) inhibitors. If steroids are utilized for induction, efforts should be made to introduce steroid-sparing agents for maintenance therapy. Of note, guidelines state corticosteroids may be avoided entirely when other effective induction strategies are planned.[10] Both guidelines also recommend that any drug that effectively treats induction should be continued for maintenance.[9,10]
- **Uveitis and Ocular Inflammatory Disorders:** American Academy of Ophthalmology (AAO) guidelines (2014) note that adalimumab may be used in patients with uveitis due to various causes (e.g., spondyloarthropathy-associated or human leukocyte antigen [HLA]-

B27-associated uveitis, JIA-associated uveitis, and other posterior uveitides and panuveitis syndromes).[12] Adalimumab should be considered second-line in vision-threatening JIA-associated uveitis when methotrexate has failed or is not tolerated (strong recommendation) and may be used as corticosteroid-sparing treatment for vision-threatening chronic uveitis from seronegative spondyloarthropathy (strong recommendation). Adalimumab may also be considered in other patients who have vision-threatening or corticosteroid-dependent disease who have failed first-line therapies. Adalimumab should be considered as a second-line immunomodulatory agent for severe ocular inflammatory conditions including chronic and severe scleritis. This was re-affirmed in separate guidelines (2018) for non-corticosteroid systemic immunomodulatory therapy in noninfectious uveitis.[20] ACR/Arthritis Federation guidelines (2019) for uveitis associated with JIA make recommendations for use of conventional systemic DMARDs and biologics. In patients with severe active chronic anterior uveitis associated with sight-threatening complications, a TNFi (monoclonal antibody) + methotrexate is recommended.[19]

Other Uses with Supportive Evidence
There are guidelines and/or published data supporting the use of adalimumab products in the following conditions:

- **Behcet's Disease:** The European Union Against Rheumatism (EULAR) recommendations (2018) include TNFis for initial or recurrent sight-threatening uveitis.[13] For patients refractory to first-line treatments (e.g., corticosteroids), TNFis are among the treatment options for mucocutaneous manifestations, venous thrombosis, severe or refractory gastrointestinal disease, and recurrent/chronic joint involvement. Recommendations for the use of TNFis in ocular inflammatory disorders from the AAO (2014) note that TNFis may be used first-line in patients with ophthalmic manifestations of Behcet's disease and for acute exacerbations of preexisting Behcet's disease.[12]
- **Pyoderma Gangrenosum:** There are currently no formal US consensus guidelines specifically dedicated to the diagnosis and management of pyoderma gangrenosum published by major bodies like the AAD. However, clinical practice is informed by several comprehensive reviews and expert recommendations. Oral prednisone is the most common initial immunosuppressant medication.[14] Other systemic therapies include conventional systemic agents (e.g., cyclosporine, methotrexate, azathioprine, cyclophosphamide, mycophenolate mofetil) and TNFis (i.e., infliximab, etanercept, and adalimumab products). Of note, it is generally recognized that conventional systemic agents have a delayed onset of action; therefore, two months or longer is considered an adequate duration of therapy to elicit a clinical response.
- **Sarcoidosis:** The European Respiratory Society Task Force (2021) has guidelines for treatment of pulmonary, cutaneous, cardiac, and neurologic sarcoidosis.[15] Adalimumab is a recommended therapy after continued disease or relapse while taking systemic corticosteroids and immunosuppressants (e.g., methotrexate, azathioprine, leflunomide, mycophenolate mofetil, hydroxychloroquine).

Coverage Policy

POLICY STATEMENT
Prior Authorization is required for benefit coverage of adalimumab products. All approvals are provided for the duration noted below. In cases where the approval is authorized in months, 1 month is equal to 30 days. Because of the specialized skills required for evaluation and diagnosis of patients treated with adalimumab products as well as the monitoring required for adverse events and long-term efficacy, initial approval requires the agent to be prescribed by or in consultation with a physician who specializes in the condition being treated.

NOTE: This product also requires the use of preferred products before approval of a requested non-preferred adalimumab product. Refer to the respective policy *Inflammatory Conditions – Adalimumab Products Preferred Specialty Management Policy for Legacy Drug List Plans (PSM003), Inflammatory Conditions – Adalimumab Products Preferred Specialty Management Policy: Standard/Performance, Value/Advantage and Total Savings Prescription Drug Lists (PSM013), or Inflammatory Conditions – Adalimumab Products Preferred Specialty Management Policy for Individual and Family Plans (PSM014)* for additional preferred product criteria requirements and exceptions.

The following products are not covered on the pharmacy and medical benefits for Employer Plans and Individual and Family Plans: adalimumab-bwwd, adalimumab-ryvk (-ryvk NDCs starting with 51759), Amjevita (NDCs starting with 84612), Humira (NDCs starting with 83457), Hyrimoz (NDCs starting with 83457). Refer to the customer's benefit plan document for details of covered product(s).

Adalimumab products are considered medically necessary when ONE of the following is met (1-14):

FDA-Approved Indications

1. **Ankylosing Spondylitis.** Approve for the duration noted if the patient meets ONE of the following (A or B):
   - **A)** Initial Therapy. Approve for 6 months if the patient meets BOTH of the following (i and ii):
     - **i.** Patient is ≥ 18 years of age; AND
     - **ii.** The medication is prescribed by or in consultation with a rheumatologist; OR
   - **B)** Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):
     - **i.** Patient has been established on therapy for at least 6 months; AND
       Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).
     - **ii.** Patient meets at least ONE of the following (a or b):
       - **a)** When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
         Note: Examples of objective measures include Ankylosing Spondylitis Disease Activity Score (ASDAS), Ankylosing Spondylitis Quality of Life Scale (ASQoL), Bath Ankylosing Spondylitis Disease Activity Index (BASDAI), Bath Ankylosing Spondylitis Functional Index (BASFI), Bath Ankylosing Spondylitis Global Score (BAS-G), Bath Ankylosing Spondylitis Metrology Index (BASMI), Dougados Functional Index (DFI), Health Assessment Questionnaire for the Spondyloarthropathies (HAQ-S), and/or serum markers (e.g., C-reactive protein, erythrocyte sedimentation rate).
       - **b)** Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain or stiffness, or improvement in function or activities of daily living.

2. **Crohn's Disease.** Approve for the duration noted if the patient meets ONE of the following (A or B):
   - **A)** Initial Therapy. Approve for 6 months if the patient meets BOTH of the following (i and ii):
     - **i.** Patient is ≥ 6 years of age; AND
     - **ii.** The medication is prescribed by or in consultation with a gastroenterologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):
- **i.** Patient has been established on therapy for at least 6 months; AND
  Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).
- **ii.** Patient meets at least ONE of the following (a or b):
  - **a)** When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
    Note: Examples of objective measures include fecal markers (e.g., fecal lactoferrin, fecal calprotectin), serum markers (e.g., C-reactive protein), imaging studies (magnetic resonance enterography [MRE], computed tomography enterography [CTE]), endoscopic assessment, and/or reduced dose of corticosteroids.
  - **b)** Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain, fatigue, stool frequency, and/or blood in stool.

3. Juvenile Idiopathic Arthritis (JIA) [or juvenile rheumatoid arthritis] {regardless of type of onset}. Approve for the duration noted if the patient meets ONE of the following (A or B):
Note: This includes a patient with juvenile spondyloarthropathy/active sacroiliac arthritis.

A) Initial Therapy. Approve for 6 months if the patient meets ALL of the following (i, ii, and iii):
- **i.** Patient is ≥ 2 years of age; AND
- **ii.** Patient meets ONE of the following (a, b, c, or d):
  - **a)** Patient has tried one other systemic therapy for this condition; OR
    Note: Examples of other systemic therapies for JIA include methotrexate, sulfasalazine, leflunomide, or a nonsteroidal anti-inflammatory drug (NSAID) [e.g., ibuprofen, naproxen]. A previous trial of one biologic other than the requested medication also counts as a trial of one systemic therapy for JIA. A biosimilar of the requested biologic does not count. Refer to Appendix for examples of biologics used for JIA.
  - **b)** Patient will be starting on adalimumab concurrently with methotrexate, sulfasalazine, or leflunomide; OR
  - **c)** Patient has an absolute contraindication to methotrexate, sulfasalazine, or leflunomide; OR
    Note: Examples of contraindications to methotrexate include pregnancy, breast feeding, alcoholic liver disease, immunodeficiency syndrome, blood dyscrasias.
  - **d)** Patient has aggressive disease, as determined by the prescriber; AND
- **iii.** The medication is prescribed by or in consultation with a rheumatologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):
- **i.** Patient has been established on therapy for at least 6 months; AND
  Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).
- **ii.** Patient meets at least ONE of the following (a or b):
  - **a)** When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
    Note: Examples of objective measures include Physician Global Assessment (MD global), Parent/Patient Global Assessment of Overall Well-Being (PGA), Parent/Patient Global Assessment of Disease Activity (PDA), Juvenile Arthritis Disease Activity Score (JDAS), Clinical Juvenile Arthritis Disease Activity Score (cJDAS), Juvenile Spondyloarthritis Disease Activity Index (JSpADA), serum

markers (e.g., C-reactive protein, erythrocyte sedimentation rate), and/or reduced dosage of corticosteroids.

b) Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as improvement in limitation of motion, less joint pain or tenderness, decreased duration of morning stiffness or fatigue, or improved function or activities of daily living.

4. Hidradenitis Suppurativa. Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 3 months if the patients meets ALL of the following (i, ii, and iii):

i. Patient is ≥ 12 years of age; AND

ii. Patient has tried at least ONE other therapy; AND
Note: Examples include intralesional or oral corticosteroids (such as triamcinolone or prednisone), systemic antibiotics (e.g., clindamycin, dicloxacillin, or erythromycin), or isotretinoin.

iii. The medication is prescribed by or in consultation with a dermatologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets ALL of the following (i, ii, and iii):

i. Patient has been established on therapy for at least 3 months; AND
Note: A patient who has received < 3 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).

ii. When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); AND
Note: Examples of objective measures include Hurley staging, Sartorius score, Physician Global Assessment, and Hidradenitis Suppurativa Severity Index.

iii. Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain or drainage of lesions, nodules, or cysts.

5. Plaque Psoriasis. Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 3 months if the patient meets ALL of the following (i, ii, and iii):

i. Patient is ≥ 18 years of age; AND

ii. Patient meets ONE of the following (a or b):

a) Patient has tried at least one traditional systemic agent for psoriasis for at least 3 months, unless intolerant; OR
Note: Examples include methotrexate, cyclosporine, or acitretin. A 3-month trial of psoralen plus ultraviolet A light (PUVA) also counts. An exception to the requirement for a trial of one traditional systemic agent for psoriasis can be made if the patient has already had a 3-month trial or previous intolerance to at least one biologic (other than the requested medication), Otezla/Otezla XR (apremilast tablets/extended-release tablets), or Sotyktu (deucravacitinib tablets). A biosimilar of the requested biologic does not count. Refer to Appendix for examples of biologics used for psoriasis. A patient who has already tried a biologic for psoriasis, Otezla/Otezla XR, or Sotyktu is not required to "step back" and try a traditional systemic agent for psoriasis.

b) According to the prescriber, the patient has a contraindication to methotrexate; AND

iii. The medication is prescribed by or in consultation with a dermatologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets ALL of the following (i, ii, and iii):

i. Patient has been established on therapy for at least 3 months; AND
Note: A patient who has received < 3 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).
ii. Patient experienced a beneficial clinical response, defined as improvement from baseline (prior to initiating an adalimumab product) in at least one of the following: estimated body surface area affected, erythema, induration/thickness, and/or scale of areas affected by psoriasis; AND
iii. Compared with baseline (prior to receiving an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain, itching, and/or burning.

6. Psoriatic Arthritis. Approve for the duration noted if the patient meets ONE of the following (A or B):
A) Initial Therapy. Approve for 6 months if the patient meets BOTH of the following (i and ii):
i. Patient is ≥ 18 years of age; AND
ii. The medication is prescribed by or in consultation with a rheumatologist or a dermatologist.
B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):
i. Patient has been established on therapy for at least 6 months; AND
Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).
ii. Patient meets at least ONE of the following (a or b):
a) When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
Note: Examples of objective measures of disease activity include Disease Activity Index for Psoriatic Arthritis (DAPSA), Composite Psoriatic Disease Activity Index (CPDAI), Psoriatic Arthritis Disease Activity Score (PsA DAS), Grace Index, Leeds Enthesitis Score (LEI), Spondyloarthritis Consortium of Canada (SPARCC) enthesitis score, Leeds Dactylitis Instrument Score, Minimal Disease Activity (MDA), Psoriatic Arthritis Impact of Disease (PsAID-12), and/or serum markers (e.g., C-reactive protein, erythrocyte sedimentation rate).
b) Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as less joint pain, morning stiffness, or fatigue; improved function or activities of daily living; or decreased soft tissue swelling in joints or tendon sheaths.

7. Rheumatoid Arthritis. Approve for the duration noted if the patient meets ONE of the following (A or B):
A) Initial Therapy. Approve for 6 months if the patient meets ALL of the following (i, ii, and iii):
i. Patient is ≥ 18 years of age; AND
ii. Patient has tried ONE conventional synthetic disease-modifying antirheumatic drug (DMARD) for at least 3 months; AND
Note: Examples include methotrexate (oral or injectable), leflunomide, hydroxychloroquine, and sulfasalazine. An exception to the requirement for a trial of one conventional synthetic DMARD can be made if the patient has already had a 3-month trial with at least one biologic other than the requested medication. A biosimilar of the requested biologic does not count. Refer to Appendix for examples of biologics used for rheumatoid arthritis. A patient who has already tried a biologic for rheumatoid arthritis is not required to "step back" and try a conventional synthetic DMARD.
ii. The medication is prescribed by or in consultation with a rheumatologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):
- **i.** Patient has been established on therapy for at least 6 months; AND
  Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).
- **ii.** Patient meets at least ONE of the following (a or b):
  - **a)** Patient experienced a beneficial clinical response when assessed by at least one objective measure; OR
    Note: Examples of objective measures of disease activity include Clinical Disease Activity Index (CDAI), Disease Activity Score (DAS) 28 using erythrocyte sedimentation rate or C-reactive protein, Patient Activity Scale (PAS)-II, Rapid Assessment of Patient Index Data 3 (RAPID-3), and/or Simplified Disease Activity Index (SDAI).
  - **b)** Patient experienced an improvement in at least one symptom, such as decreased joint pain, morning stiffness, or fatigue; improved function or activities of daily living; or decreased soft tissue swelling in joints or tendon sheaths.

8. Ulcerative Colitis. Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 6 months if the patient meets BOTH of the following (i and ii):
- **i.** Patient is ≥ 5 years of age; AND
- **ii.** The medication is prescribed by or in consultation with a gastroenterologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):
- **i.** Patient has been established on therapy for at least 6 months; AND
  Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).
- **ii.** Patient meets at least ONE of the following (a or b):
  - **a)** When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
    Note: Examples of objective measures include fecal markers (e.g., fecal calprotectin), serum markers (e.g., C-reactive protein), endoscopic assessment, and/or reduced dose of corticosteroids.
  - **b)** Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain, fatigue, stool frequency, and/or rectal bleeding.

9. Uveitis (including other posterior uveitides and panuveitis syndromes). Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 6 months if the patient meets ALL of the following (i, ii, and iii):
- **i.** Patient is ≥ 2 years of age; AND
- **ii.** Patient has tried ONE of the following therapies: periocular, intraocular, or systemic corticosteroids; immunosuppressives; AND
  Note: Examples of corticosteroids include prednisolone, triamcinolone, betamethasone, methylprednisolone, and prednisone. Examples of immunosuppressive agents include methotrexate, mycophenolate mofetil, azathioprine, and cyclosporine. A trial of one biologic other than the requested medication also counts. A biosimilar of the requested biologic does not count.
- **iii.** The medication is prescribed by or in consultation with an ophthalmologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):
- **i.** Patient has been established on therapy for at least 6 months; AND

Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).

ii. Patient meets at least one of the following (a or b):

a) When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
Note: Examples of objective measures include best-corrected visual acuity, assessment of chorioretinal and/or inflammatory retinal vascular lesions, or anterior chamber cell grade or vitreous haze grade.

b) Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased eye pain, redness, light sensitivity, and/or blurred vision; or improvement in visual acuity.

Other Uses with Supportive Evidence

10. Behcet's Disease. Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 3 months if the patient meets ALL of the following (i, ii, and ii):

i. Patient is ≥ 2 years of age; AND

ii. Patient meets ONE of the following (a or b):

a) Patient has tried at least ONE conventional therapy; OR
Note: Examples include systemic corticosteroids (e.g., methylprednisolone), immunosuppressants (e.g., azathioprine, methotrexate, mycophenolate mofetil, cyclosporine, tacrolimus, Leukeran [chlorambucil tablets], cyclophosphamide, interferon alfa, or Otezla/Otezla XR (apremilast tablets/extended-release tablets). A trial of one biologic other than the requested medication also counts. A patient who has already tried one biologic other than the requested drug for Behcet's disease is not required to "step back" and try a conventional therapy. A biosimilar of the requested biologic does not count.

b) Patient has ophthalmic manifestations of Behcet's disease; AND

iii. The medication is prescribed by or in consultation with a rheumatologist, dermatologist, ophthalmologist, gastroenterologist, or neurologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets ALL of the following (i, ii, and iii):

i. Patient has been established on therapy for at least 3 months; AND
Note: A patient who has received < 3 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).

ii. When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); AND
Note: Examples of objective measures are dependent upon organ involvement but may include best-corrected visual acuity (if ophthalmic manifestations); serum markers (e.g., C-reactive protein, erythrocyte sedimentation rate); or ulcer depth, number, and/or lesion size.

iii. Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain or improved visual acuity (if ophthalmic manifestations).

11. Pyoderma Gangrenosum. Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 4 months if the patient meets ALL of the following (i, ii, and iii):

i. Patient is ≥ 18 years of age; AND

ii. Patient meets ONE of the following (a or b):

a) Patient has tried one systemic corticosteroid; OR
Note: An example is prednisone.
b) Patient has tried one other immunosuppressant for at least 2 months or was intolerant to one of these agents; AND
Note: Examples include mycophenolate mofetil and cyclosporine.

iii. The medication is prescribed by or in consultation with a dermatologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets ALL of the following (i, ii, and iii):

i. Patient has been established on therapy for at least 4 months; AND
Note: A patient who has received < 4 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).

ii. Patient experienced a beneficial clinical response, defined as improvement from baseline (prior to initiating an adalimumab product) in at least one of the following: size, depth, and/or number of lesions; AND

iii. Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain and/or tenderness of affected lesions.

12. Sarcoidosis. Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 3 months if the patient meets ALL of the following (i, ii, iii, and iv):

i. Patient is ≥ 18 years of age; AND

ii. Patient has tried at least one corticosteroid; AND
Note: An example is prednisone.

iii. Patient has tried at least one immunosuppressive medication; AND
Note: Examples include methotrexate, leflunomide, azathioprine, mycophenolate mofetil, hydroxychloroquine, an infliximab product, or chloroquine.

iv. The medication is prescribed by or in consultation with a pulmonologist, ophthalmologist, cardiologist, neurologist, or dermatologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets ALL of the following (i, ii, and iii):

i. Patient has been established on therapy for at least 3 months; AND
Note: A patient who has received < 3 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).

ii. When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); AND
Note: Examples of objective measures are dependent upon organ involvement but may include lung function (e.g., predicted forced vital capacity and/or 6-minute walk distance); serum markers (e.g., C-reactive protein, liver enzymes, N-terminal pro-brain natriuretic peptide [NT-proBNP]); improvement in rash or skin manifestations, neurologic symptoms, or rhythm control; or imaging (e.g., if indicated, chest radiograph, magnetic resonance imaging [MRI], or echocardiography).

iii. Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased cough, fatigue, pain, palpitations, neurologic symptoms, and/or shortness of breath.

13. Scleritis or Sterile Corneal Ulceration. Approve for the duration noted if the patient meets ONE of the following (A or B):

A) Initial Therapy. Approve for 6 months if the patient meets ALL of the following (i, ii, and iii):

i. Patient is ≥ 18 years of age; AND

ii. Patient has tried one other therapy for this condition; AND

Note: Examples include oral nonsteroidal anti-inflammatory drugs (NSAIDs) such as indomethacin, naproxen, or ibuprofen; oral, topical (ophthalmic), or intravenous corticosteroids (such as prednisone, prednisolone, methylprednisolone); methotrexate; cyclosporine; or other immunosupressants.

iii. The medication is prescribed by or in consultation with an ophthalmologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):

i. Patient has been established on therapy for at least 6 months; AND
Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).

ii. Patient meets at least ONE of the following (a or b):

a) When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
Note: Examples of objective measures are serum markers (e.g., C-reactive protein, erythrocyte sedimentation rate).

b) Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased eye pain, redness, light sensitivity, tearing, and/or improvement in visual acuity.

14. Spondyloarthritis, Other Subtypes. Approve for the duration noted if the patient meets ONE of the following (A or B):
Note: This includes undifferentiated arthritis, non-radiographic axial spondyloarthritis, reactive arthritis (Reiter's disease), or arthritis associated with inflammatory bowel disease. For ankylosing spondylitis or psoriatic arthritis, refer to the respective criteria under FDA-approved indications.

A) Initial Therapy. Approve for 6 months if the patient meets ALL of the following (i, ii, and iii):

i. Patient is ≥ 18 years of age; AND

ii. Patient meets ONE of the following (a or b):

a) Patient meets BOTH of the following [(1) and (2)]:

(1) Patient has arthritis primarily in the knees, ankles, elbows, wrists, hands, and/or feet; AND

(2) Patient has tried at least one conventional synthetic disease-modifying antirheumatic drug (DMARD); OR
Note: Examples include methotrexate, leflunomide, or sulfasalazine.

b) Patient has axial spondyloarthritis AND has objective signs of inflammation, defined as at least ONE of the following [(1) or (2)]:

(1) C-reactive protein elevated beyond the upper limit of normal for the reporting laboratory; OR

(2) Sacroiliitis reported on magnetic resonance imaging (MRI); AND

iii. The medication is prescribed by or in consultation with a rheumatologist; OR

B) Patient is Currently Receiving an Adalimumab Product. Approve for 1 year if the patient meets BOTH of the following (i and ii):

i. Patient has been established on therapy for at least 6 months; AND
Note: A patient who has received < 6 months of therapy or who is restarting therapy with an adalimumab product is reviewed under criterion A (Initial Therapy).

ii. Patient meets at least ONE of the following (a or b):

a) When assessed by at least one objective measure, patient experienced a beneficial clinical response from baseline (prior to initiating an adalimumab product); OR
Note: Examples of objective measures include Ankylosing Spondylitis Disease Activity Score (ASDAS) and/or serum markers (e.g., C-reactive protein, erythrocyte sedimentation rate).

b) Compared with baseline (prior to initiating an adalimumab product), patient experienced an improvement in at least one symptom, such as decreased pain or stiffness, or improvement in function or activities of daily living.

Conditions Not Covered

Adalimumab products for any other use are considered not medically necessary, including the following (this list may not be all inclusive; criteria will be updated as new published data are available):

1. **Concurrent Use with a Biologic or with a Targeted Synthetic Oral Small Molecule Drug.** This medication should not be administered in combination with another biologic or with a targeted synthetic oral small molecule drug used for an inflammatory condition (see Appendix for examples). Combination therapy is generally not recommended due to a potentially higher rate of adverse events and lack of controlled clinical data supporting additive efficacy.
   Note: This does NOT exclude the use of conventional synthetic disease-modifying antirheumatic drug(s) [DMARDs] (e.g., methotrexate, leflunomide, hydroxychloroquine, or sulfasalazine) in combination with this medication.

2. **Polymyalgia Rheumatica (PMR).** EULAR/ACR guidelines for the management of PMR (2015) strongly recommend against the use of TNFis for treatment of PMR.[17] This recommendation is based on lack of evidence for benefit as well as considerable potential for harm.

Coding Information

Note: 1) This list of codes may not be all-inclusive.
2) Deleted codes and codes which are not effective at the time the service is rendered may not be eligible for reimbursement.

Considered Medically Necessary when criteria in the applicable policy statements listed above are met:

HCPCS Codes	Description
C9399	Unclassified drugs or biologicals
J0139	Injection, adalimumab, 1 mg
J3490	Unclassified drugs
J3590	Unclassified biologics
Q5140	Injection, adalimumab-fkjp, biosimilar, 1 mg
Q5141	Injection, adalimumab-aaty, biosimilar, 1 mg
Q5142	Injection, adalimumab-ryvk biosimilar, 1 mg
Q5143	Injection, adalimumab-adbm, biosimilar, 1 mg
Q5144	Injection, adalimumab-aacf (Idacio), biosimilar, 1 mg
Q5145	Injection, adalimumab-afzb (Abrilada), biosimilar, 1 mg

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Revision Details

Summary of Changes	Review Date	Effective Date
New policy	09/12/2024	11/01/2024
The following coding changes were made: **Updated HCPCS Coding:** **Added:** C9399, J0139, J3490, J3590 **Added Codes Effective 1/1/2025:** Q5140, Q5141, Q5142, Q5143, Q5144, Q5145	Not Applicable-Coding Update Only	2/15/2025
No criteria changes	04/03/2025	05/01/2025
Adalimumab-aaty (unbranded Yuflyma) was added to the policy. The same criteria apply as the other adalimumab subcutaneous products.	05/08/2025	06/01/2025
Ulcerative Colitis: For initial therapy, removed the following options of approval: (1) the patient has tried one systemic therapy; (2) the patient has pouchitis and tried an antibiotic, probiotic, corticosteroid enema, or mesalamine enema.	07/31/2025	09/01/2025
Plaque Psoriasis: For initial therapy, in the Note, a 3-month trial or prior intolerance to Otezla/Otezla XR (apremilast tablets/extended-release tablets) or Sotyktu (deucravacitinib tablets) was added as an exception to the requirement for a trial of one traditional systemic agent for psoriasis. For initial therapy, the requirement "patient has a contraindication to methotrexate, as determined by the prescriber" was modified to "according to the prescriber, the patient has a contraindication to methotrexate". In the Appendix, Otezla XR (apremilast extended-release tablets) was added under the Oral Therapies/Targeted Synthetic Oral Small Molecular Drugs.	11/13/2025	01/01/2026
Adalimumab-bwwd was added to the policy. A note was added for adalimumab-bwwd, adalimumab-ryvk (-ryvk NDCs starting with 51759), Amjevita (NDCs starting with 84612), Humira (NDCs starting with 83457), Hyrimoz (NDCs starting with 83457) to designate them as Non-Covered products for Employer Plans and Individual and Family Plans.	01/29/2026	03/01/2026
Crohn's Disease: For initial therapy, the following options of approval were removed: The patient has tried or is currently taking systemic corticosteroids, or corticosteroids are contraindicated; patient has tried one	02/19/2026	03/15/2026

conventional systemic therapy for Crohn's disease along with the associated note; patient has enterocutaneous (perianal or abdominal) or rectovaginal fistulas; patient had ileocolonic resection (to reduce the chance of Crohn's disease recurrence).		
Behcet's Disease: Otezla/Otezla XR (apremilast tablets/extended-release tablets) added as an example of conventional therapy. **Sarcoidosis**: Cylosporine, Leukeran (chlorambucil tablets), cyclophosphamide, and Thalomid (thalidomide capsules) were removed, and hydroxychloroquine was added as examples of immunosuppressive medications. Cardiologist and neurologist were added as accepted specialists to the specialist requirement. **Appendix:** Sotyktu was updated to include psoriatic arthritis indication.	04/02/2026	05/01/2026

The policy effective date is in force until updated or retired.

APPENDIX

	Mechanism of Action	Examples of Indications*
Biologics		
Adalimumab SC Products (Humira®, biosimilars)	Inhibition of TNF	AS, CD, JIA, PsO, PsA, RA, UC
Cimzia® (certolizumab pegol SC injection)	Inhibition of TNF	AS, CD, nr-axSpA, PsO, PsA, RA
Etanercept SC Products (Enbrel®, biosimilars)	Inhibition of TNF	AS, JIA, PsO, PsA, RA
Infliximab IV Products (Remicade®, biosimilars)	Inhibition of TNF	AS, CD, PsO, PsA, RA, UC
Zymfentra® (infliximab-dyyb SC injection)	Inhibition of TNF	CD, UC
Simponi®, Simponi Aria® (golimumab SC injection, golimumab IV infusion)	Inhibition of TNF	SC formulation: AS, PsA, RA, UC
		IV formulation: AS, PJIA, PsA, RA
Tocilizumab Products (Actemra® IV, biosimilar; Actemra SC, biosimilar)	Inhibition of IL-6	SC formulation: PJIA, RA, SJIA
		IV formulation: PJIA, RA, SJIA
Kevzara® (sarilumab SC injection)	Inhibition of IL-6	RA
Orencia® (abatacept IV infusion, abatacept SC injection)	T-cell costimulation modulator	SC formulation: JIA, PSA, RA
		IV formulation: JIA, PsA, RA
Rituximab IV Products (Rituxan®, biosimilars)	CD20-directed cytolytic antibody	RA
Kineret® (anakinra SC injection)	Inhibition of IL-1	JIA^, RA
Omvoh® (mirikizumab IV infusion, SC injection)	Inhibition of IL-23	CD, UC
Ustekinumab Products (Stelara® SC, biosimilars; ustekinumab IV infusion)	Inhibition of IL-12/23	SC formulation: CD, PsO, PsA, UC
		IV formulation: CD, UC
Siliq® (brodalumab SC injection)	Inhibition of IL-17	PsO

Cosentyx® (secukinumab SC injection; secukinumab IV infusion)	Inhibition of IL-17A	SC formulation: AS, ERA, nr-axSpA, PsO, PsA
		IV formulation: AS, nr-axSpA, PsA
Taltz® (ixekizumab SC injection)	Inhibition of IL-17A	AS, nr-axSpA, PsO, PsA
Bimzelx® (bimekizumab-bkzx SC injection)	Inhibition of IL-17A/17F	PsO, AS, nr-axSpA, PsA
Ilumya® (tildrakizumab-asmn SC injection)	Inhibition of IL-23	PsO
Skyrizi® (risankizumab-rzaa SC injection, risankizumab-rzaa IV infusion)	Inhibition of IL-23	SC formulation: CD, PSA, PsO, UC
		IV formulation: CD, UC
Tremfya® (guselkumab SC injection, guselkumab IV infusion)	Inhibition of IL-23	SC formulation: PsA, PsO, UC
		IV formulation: UC
Entyvio® (vedolizumab IV infusion, vedolizumab SC injection)	Integrin receptor antagonist	CD, UC
Oral Therapies/Targeted Synthetic Oral Small Molecule Drugs		
Otezla® (apremilast tablets)	Inhibition of PDE4	PsO, PsA
Otezla XR™ (apremilast extended-release tablets)	Inhibition of PDE4	PsO, PsA
Cibinqo™ (abrocitinib tablets)	Inhibition of JAK pathways	AD
Olumiant® (baricitinib tablets)	Inhibition of JAK pathways	RA, AA
Litfulo® (ritlecitinib capsules)	Inhibition of JAK pathways	AA
Leqselvi® (deuruxolitinib tablets)	Inhibition of JAK pathways	AA
Rinvoq® (upadacitinib extended-release tablets)	Inhibition of JAK pathways	AD, AS, nr-axSpA, RA, PsA, UC
Rinvoq® LQ (upadacitinib oral solution)	Inhibition of JAK pathways	PsA, PJIA
Sotyktu® (deucravacitinib tablets)	Inhibition of TYK2	PsO, PsA
Xeljanz® (tofacitinib tablets/oral solution)	Inhibition of JAK pathways	RA, PJIA, PsA, UC
Xeljanz® XR (tofacitinib extended-release tablets)	Inhibition of JAK pathways	RA, PsA, UC
Zeposia® (ozanimod tablets)	Sphingosine 1 phosphate receptor modulator	UC
Velsipity® (etrasimod tablets)	Sphingosine 1 phosphate receptor modulator	UC

* Not an all-inclusive list of indications. Refer to the prescribing information for the respective agent for FDA-approved indications; SC – Subcutaneous; TNF – Tumor necrosis factor; AS – Ankylosing spondylitis; CD – Crohn's disease; JIA – Juvenile idiopathic arthritis; PsO – Plaque psoriasis; PsA – Psoriatic arthritis; RA – Rheumatoid arthritis; UC – Ulcerative colitis; nr-axSpA – Non-radiographic axial spondyloarthritis; IV – Intravenous, PJIA – Polyarticular juvenile idiopathic arthritis; IL – Interleukin; SJIA – Systemic juvenile idiopathic arthritis; ^ Off-label use of Kineret in JIA supported in guidelines; ERA – Enthesitis-related arthritis; DMARD – Disease-modifying antirheumatic drug; PDE4 – Phosphodiesterase 4; JAK – Janus kinase; AD – Atopic dermatitis; AA – Alopecia areata; TYK2 – Tyrosine kinase 2.

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